



IntealthTM

Advancing the Global Health Workforce

ECFMG FAIMER

Filling the Gap: International Medical Graduates, the U.S. Physician Shortage, and the Pressures Shaping the Pipeline

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Disclosures

Nothing To Disclose

Today's Discussion

- About Intealth, ECFMG, and FAIMER
- IMGs in U.S. Health Care
- A Look at the Current IMG Pipeline
- Some Pressures Shaping the IMG Pipeline
- Intealth as a Resource
- Q&A

About Intealth, ECFMG, and FAIMER

Our Organization at a Glance

Intealth

Founded 2021

Integrates ECFMG and FAIMER under one strategic framework.

- Private, non-profit
- Exchange Visitor Sponsorship Program (J-1 visa sponsorship)
- Government & Public Affairs

www.intealth.org

ECFMG

Founded 1956

Primary source verifies credentials and certifies IMGs for entry into U.S. GME. The gateway to U.S. training for 70 years.

- Division of Intealth
- ECFMG Certification
- Primary-source medical credentials verification
- Certification Verification Service

www.ecfm.org

FAIMER

Founded 2001

Strengthens health professions education globally through faculty development, research, and international partnerships.

- Division of Intealth
- Health Professions Education
- Research
- Data Resources
- IMG Support

www.faimer.org

IMGs: Rigorously Screened

**70
years**

ECFMG Certification – Gateway to U.S. GME

Requires passing USMLE Steps 1 & 2CK — the same exams as U.S. grads — demonstration of clinical skills and English language proficiency, and primary-source verification of medical education credentials.

**50+
years**

Intealth J-1 Visa Sponsorship – Ongoing Monitoring Throughout Training

Coordination among physician and teaching institution; tracking via SEVIS, a U.S. government database.

**5+
layers of vetting**

Multiple Checkpoints – Layers of Vetting Before IMG Trains in U.S.

IMGs must complete ECFMG Certification, obtain state medical licensure, have ACGME program oversight, AND clear visa processes and Intealth monitoring (if J-1). This is not a light touch.

International Medical Graduates (IMGs)

Integral to U.S. Health Care

Who is an IMG?

An international medical graduate (IMG) is any physician who received his/her basic medical degree from a medical school outside of the United States, **regardless of country of citizenship**

This includes:

- **Any U.S. citizen** who received a medical degree from a school outside of the United States
- **Foreign nationals** who received a medical degree outside of the United States
and require a visa to participate in U.S. GME

Graduates of Canadian medical schools after July 1, 2025 are considered IMGs

IMGs: Essential to the U.S. Health System

23%

of all active U.S. physicians are IMGs — physicians who received their basic medical qualification outside the United States.

Workforce backbone — The U.S. faces a projected shortage of 21,000–55,000 primary care physicians by 2033. IMGs absorb a critical share of this gap, particularly in primary care and underserved communities.

Quality of care — IMGs provide high-quality care on par with U.S.-trained physicians, with strong patient outcomes and a critical role in delivering care, especially in underserved communities.

Geographic reach — In Primary Care Service Areas where 30%+ of residents live below the poverty line, nearly 1 in 3 doctors is educated abroad. 83.7 million Americans live in primary care shortage areas.

Cultural concordance — IMGs bring experience with global disease and share language, ethnicity, and/or cultural backgrounds with diverse patient populations, improving care delivery.

IMGs In Training — Front-line Caregivers

770+

of 1,000+ U.S.
teaching hospitals
match with IMGs

23.6%

of all PGY-1
residency positions
filled by IMGs (2026)

42%

of internal medicine
positions filled
by IMGs

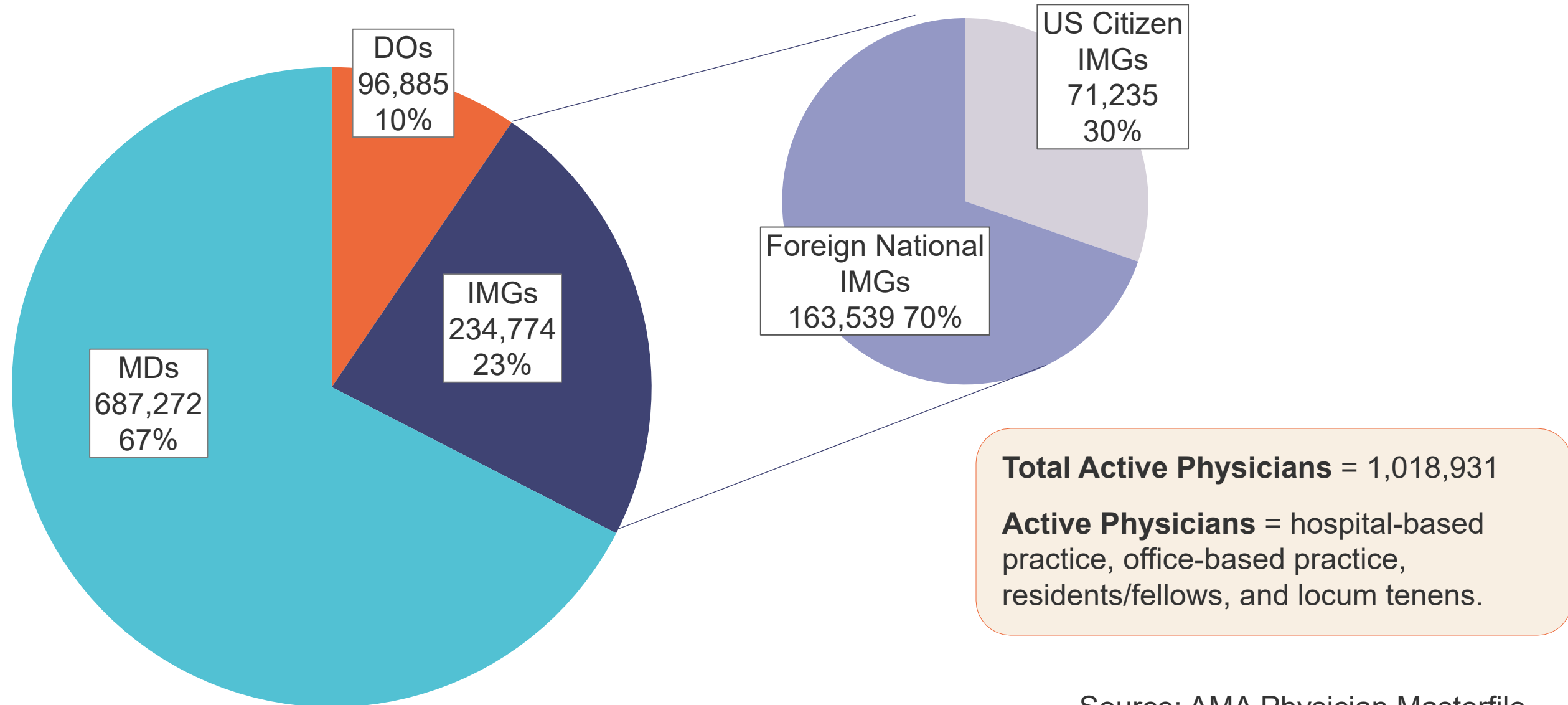
6,733

non-U.S. IMGs
matched to
residency in 2026

Residents are front-line caregivers, not trainees in the background:

- Residents provide supervised patient care in hospital units and ICUs, emergency departments, outpatient clinics, and specialty services.
- Teaching hospitals serve a disproportionate share of Medicaid and uninsured patients, the populations most dependent on the resident workforce.
- When residency positions go unfilled, existing residents absorb additional workload, potentially reducing both care quality and training quality.
- J-1 physicians are front-line caregivers for millions of Americans.

IMGs in Active Practice — A Structural Necessity

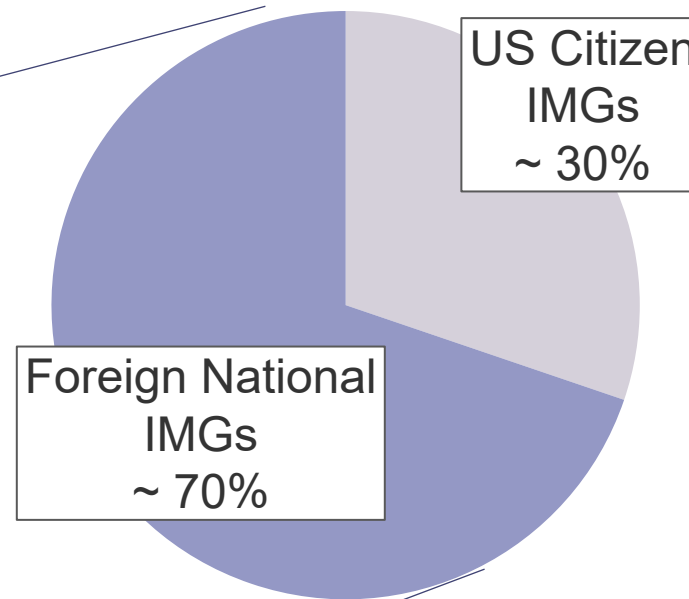


Source: AMA Physician Masterfile,

IMGs in U.S. Active Physician Workforce

Top Countries of Citizenship

- 2 India
- 3 Pakistan
- 4 Philippines
- 5 Canada

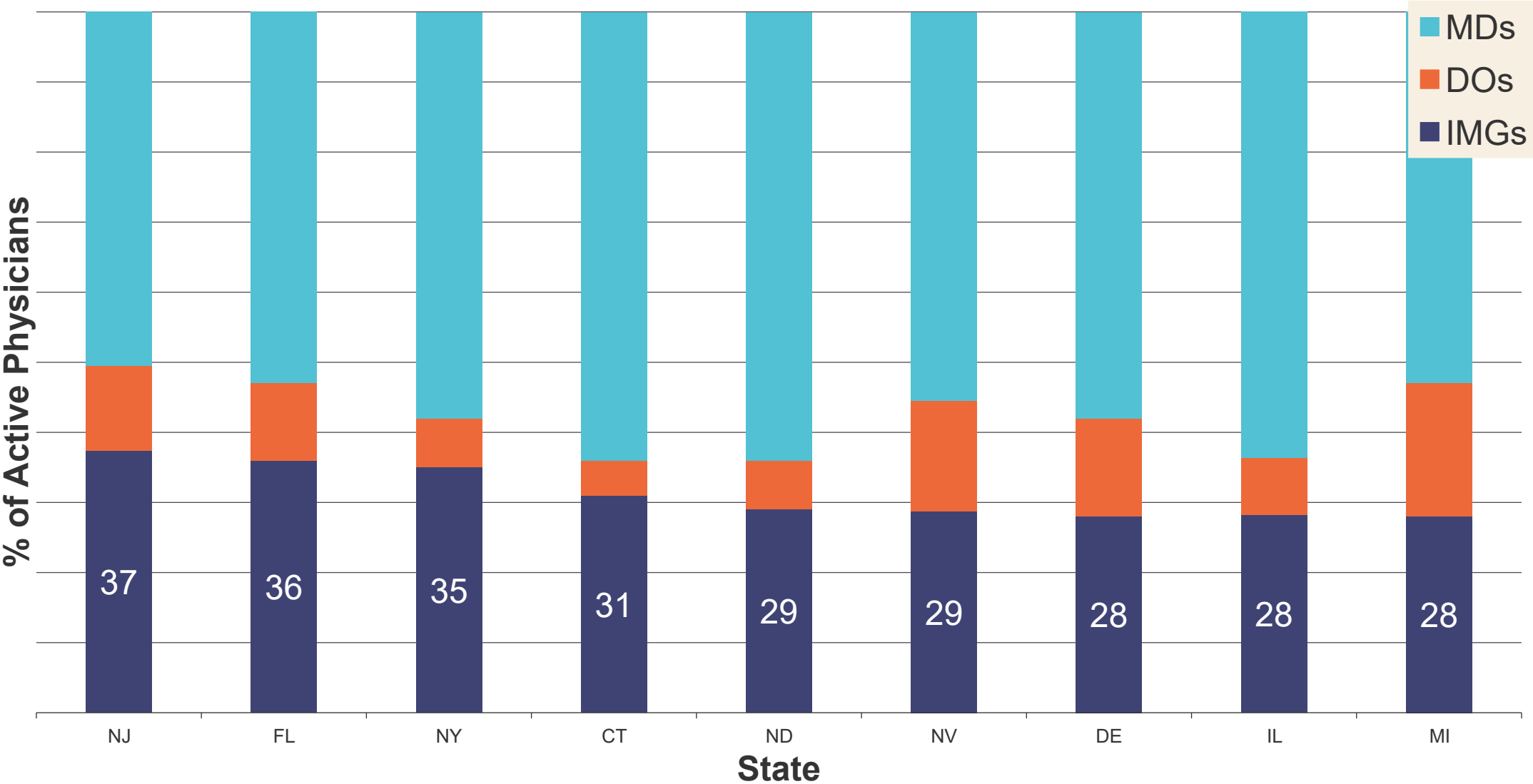


1 United States

Active Physicians = hospital-based practice, office-based practice, residents/fellows, and locum tenens.

Citizenship=At entry to medical school

Top States for IMGs, % of Active Physicians



Source: AMA Physician Masterfile, 2025

IMGs in U.S. Health Care — Primary Care

30.5%

of Primary Care Physicians

Active Primary Care Physicians=331,336

22.3%

of Physicians in SVI-designated
“Medium-high” and “High” locations

Active Physicians=317,663

Active Physicians = hospital-based practice, office-based practice, residents/fellows

Primary Care=Family Medicine, General Practice, Geriatric Medicine, Internal Medicine, Internal Medicine/Family Practice, Internal Medicine/Pediatrics, Pediatrics

Source: AMA Physician Masterfile,

2025

America Faces a Growing Doctor Shortage

86,000

projected physician
shortfall by 2036
(AAMC)

187,000

FTE physician
shortfall by 2037
(HRSA)

7,749

federally designated
primary-care
shortage areas

The gap between supply and demand is structural, not temporary:

- U.S. medical schools graduate **~32,000 students** annually, but residency programs offer **44,000+ positions**; IMGs fill the gap.
- Shortages are most acute in primary care: internal medicine, family medicine, pediatrics, and geriatrics.
- Rural and underserved communities face the deepest deficits; these are the communities J-1 physicians disproportionately serve.
- The AMA estimates each physician supports 17 jobs and generates \$3.2 million in annual economic output.

A Look at the Current IMG Pipeline

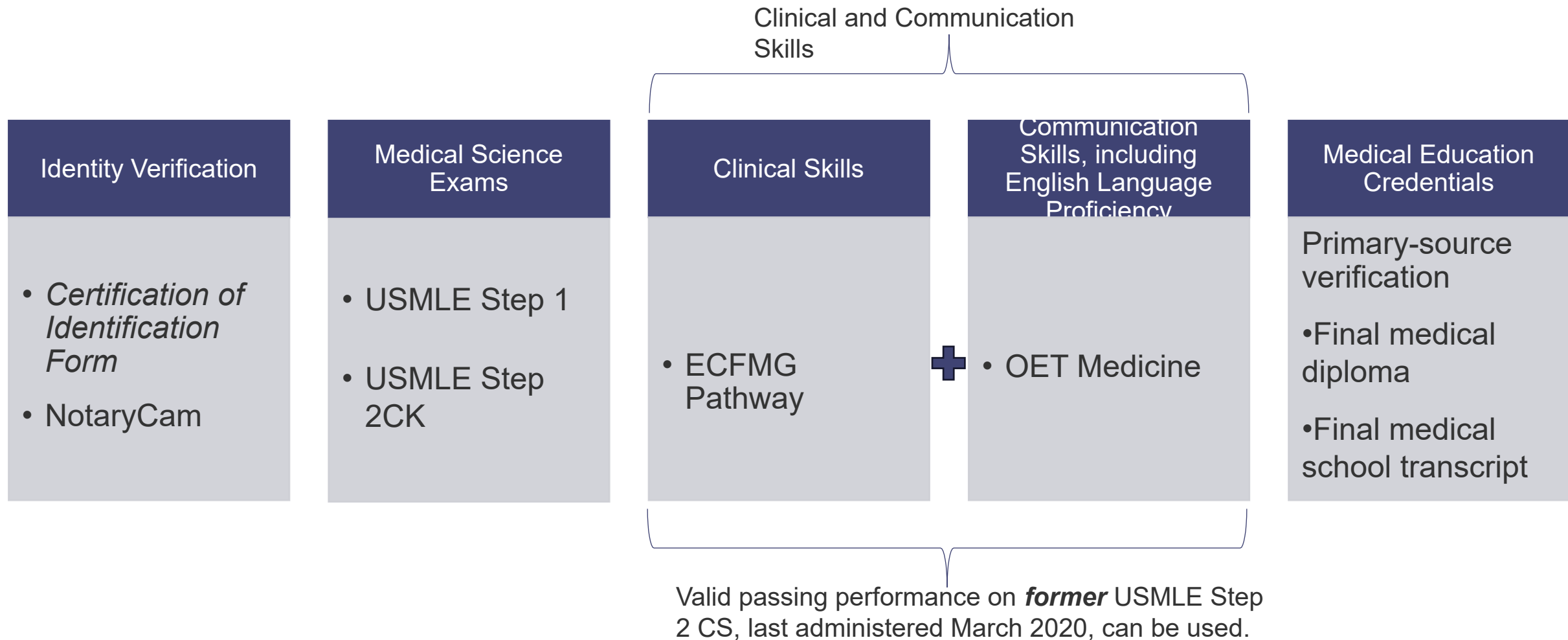
*ECFMG Certification, Match Performance,
and J-1 Visa Sponsorship*

ECFMG Certification is the Gateway to the United States for IMGs

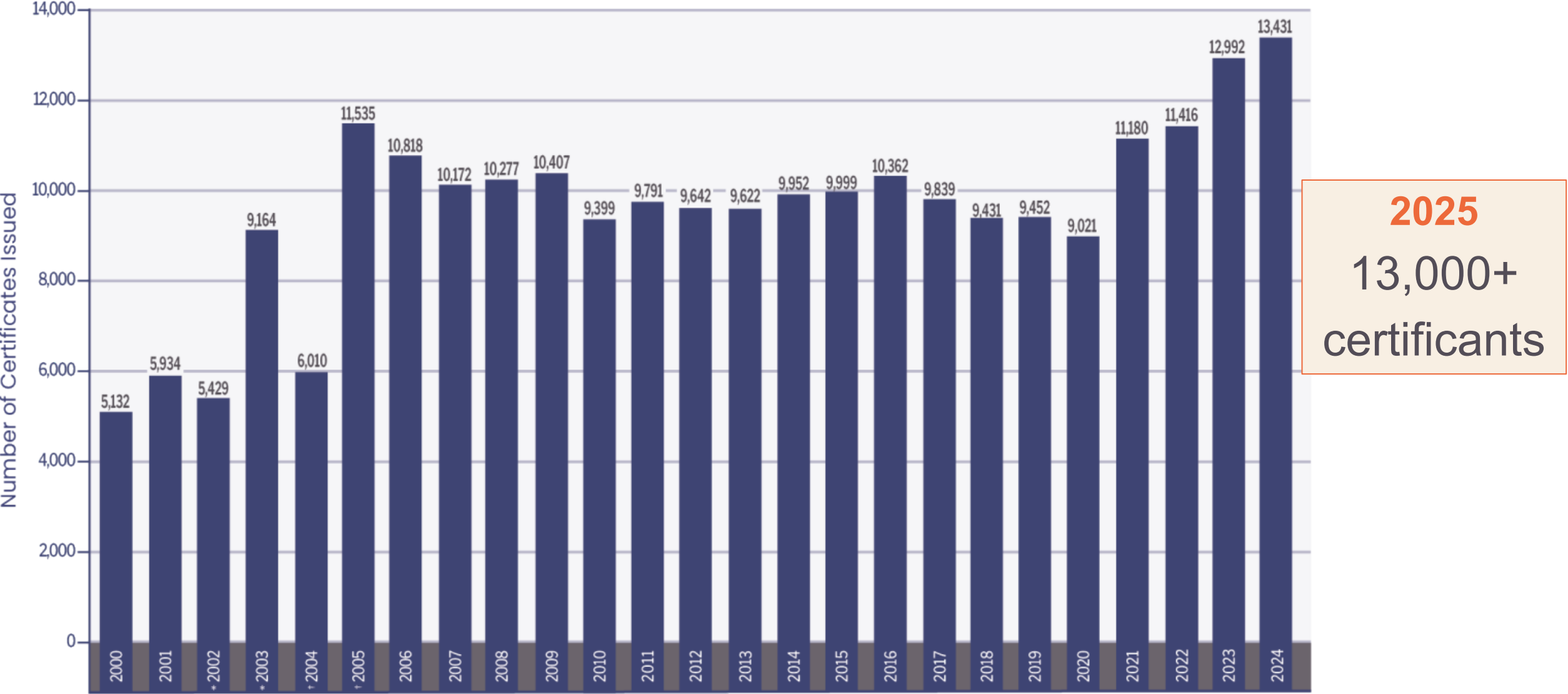
- 65+ Years Certifying IMGs: More than 420,000 physicians have been ECFMG Certified since 1956
- ECFMG Certification is required for:
 - Participation in U.S. graduate medical education (GME)
 - Step 3 of United States Licensing Exam[®] (USMLE[®])
 - Medical licensure



ECFMG Certification - Requirements



ECFMG Certificants, 2000-2024



2026 Main Residency Match: IMGs at a Glance

44,344

total positions
(all-time high)

53,373

registered applicants
(all-time high)

2,862

unfilled positions
(+15.7% vs. 2025)

6,733

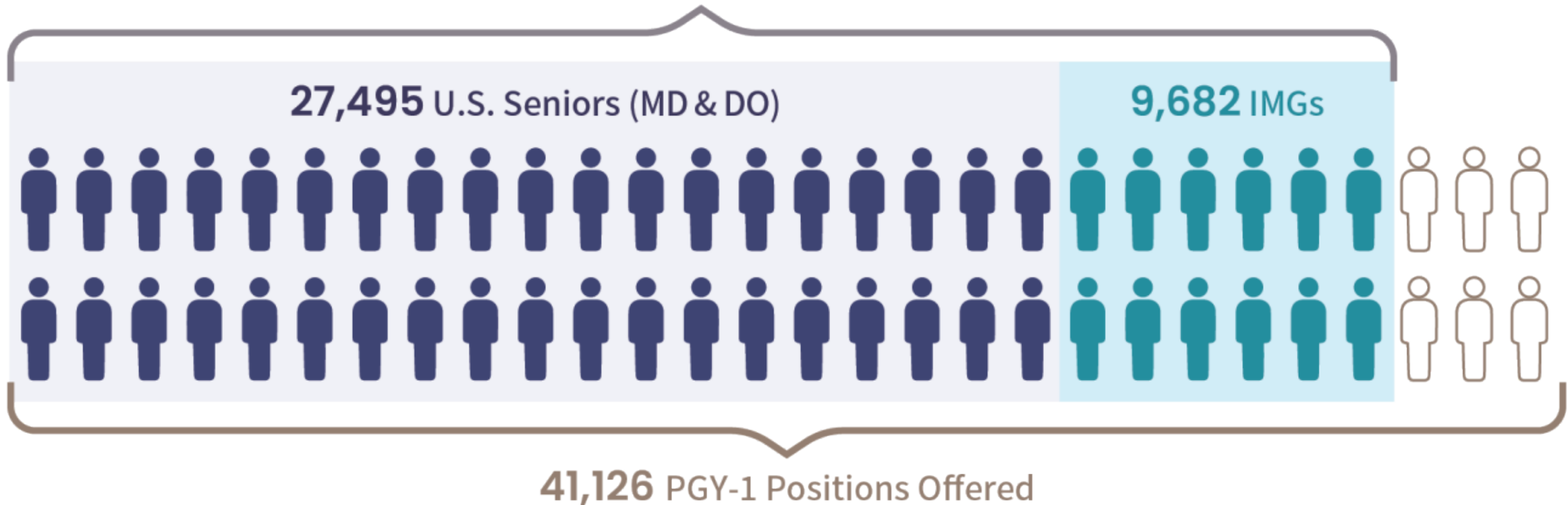
non-U.S. IMGs
matched

What the data show:

- There are more residency positions than U.S. medical graduates can fill; this is structural, not cyclical. IMGs are not an alternative; they are a necessity.
- 2,862 positions went unfilled despite record applicant volume; match rates, not applicant supply, are the constraint. Many unfilled positions are in shortage specialties.
- Specialty matches for non-U.S. IMGs are highest in high-need fields: Internal Medicine 42%, Pathology 34%, Pediatrics 30%, Neurology 22%, Psychiatry 16%.
- According to the NRMP, most U.S. MD and DO graduates match into one of their top three residency choices.

IMG Performance in the 2026 Match

PGY-1 Positions Filled

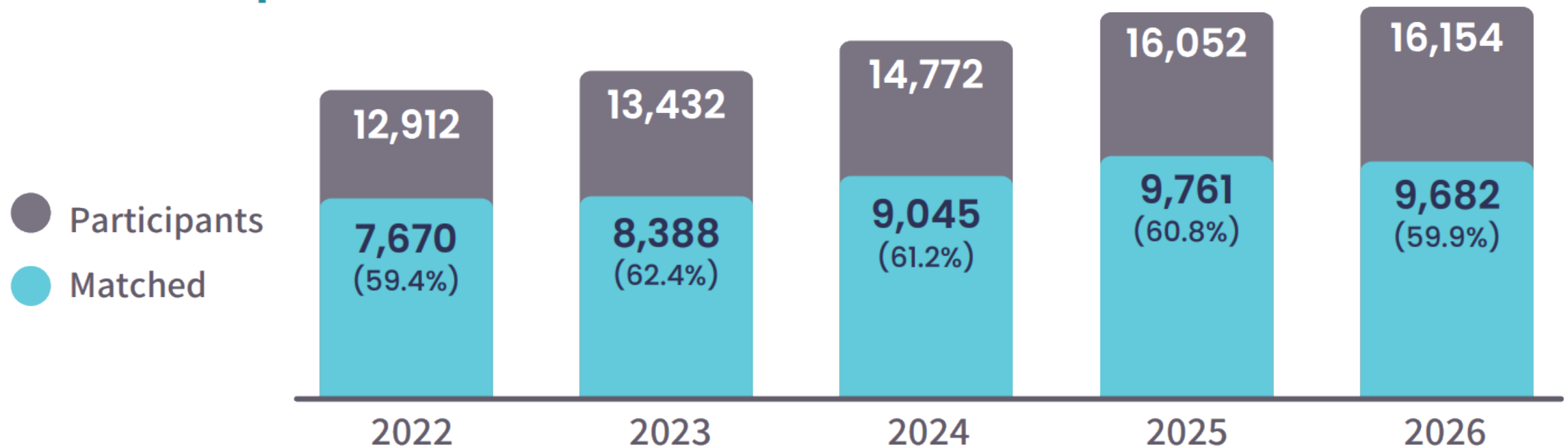


IMGs in the 2026 Main Residency Match®

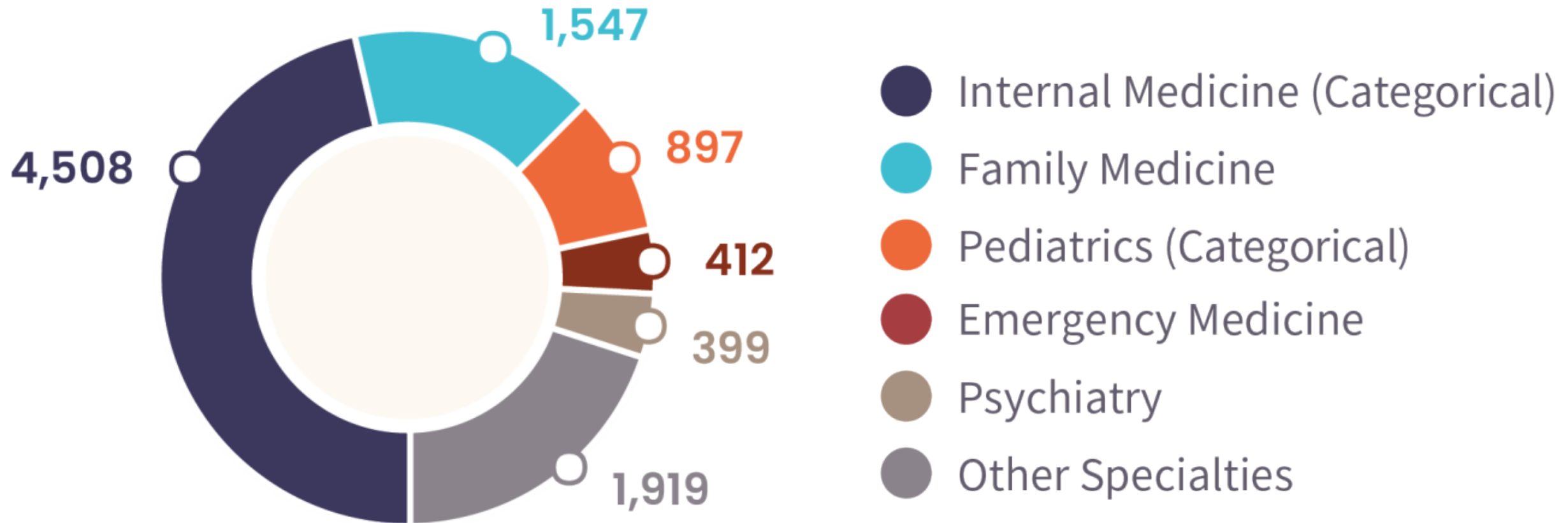
IMGs accounted for approximately 23.6% of all matched applicants.

Non-U.S. citizen IMGs reached a record absolute high of 6,733

IMG Participants and IMGs Matched



IMG Performance in the 2026 Match



Exchange Visitor (J-1 Visa) Sponsorship

- Intealth is designated by U.S. Department of State as **visa sponsor** for all J-1 physicians (~70% of foreign national trainees)
- The J-1 visa is a **temporary, non-immigrant visa** reserved for **educational and cultural exchange**
- All J-1 physicians sponsored by Intealth are subject to a **two-year home country residency requirement** following completion of training before being eligible for certain other U.S. visa classifications

About J-1 Physicians *(physicians sponsored on a J-1 visa)*

Nearly

17,500

physicians in residency and fellowship programs

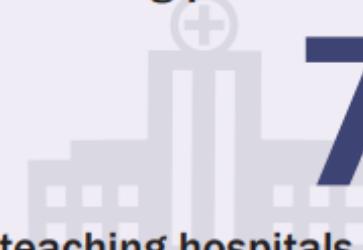
From more than



150

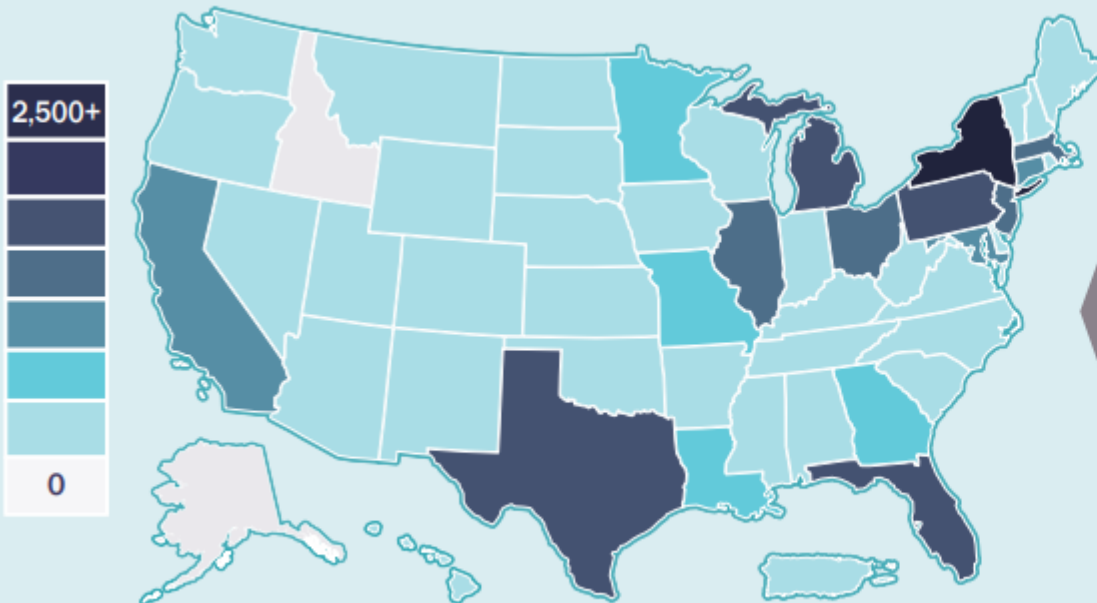
different countries of origin

Serving patients at nearly



780

teaching hospitals across the nation



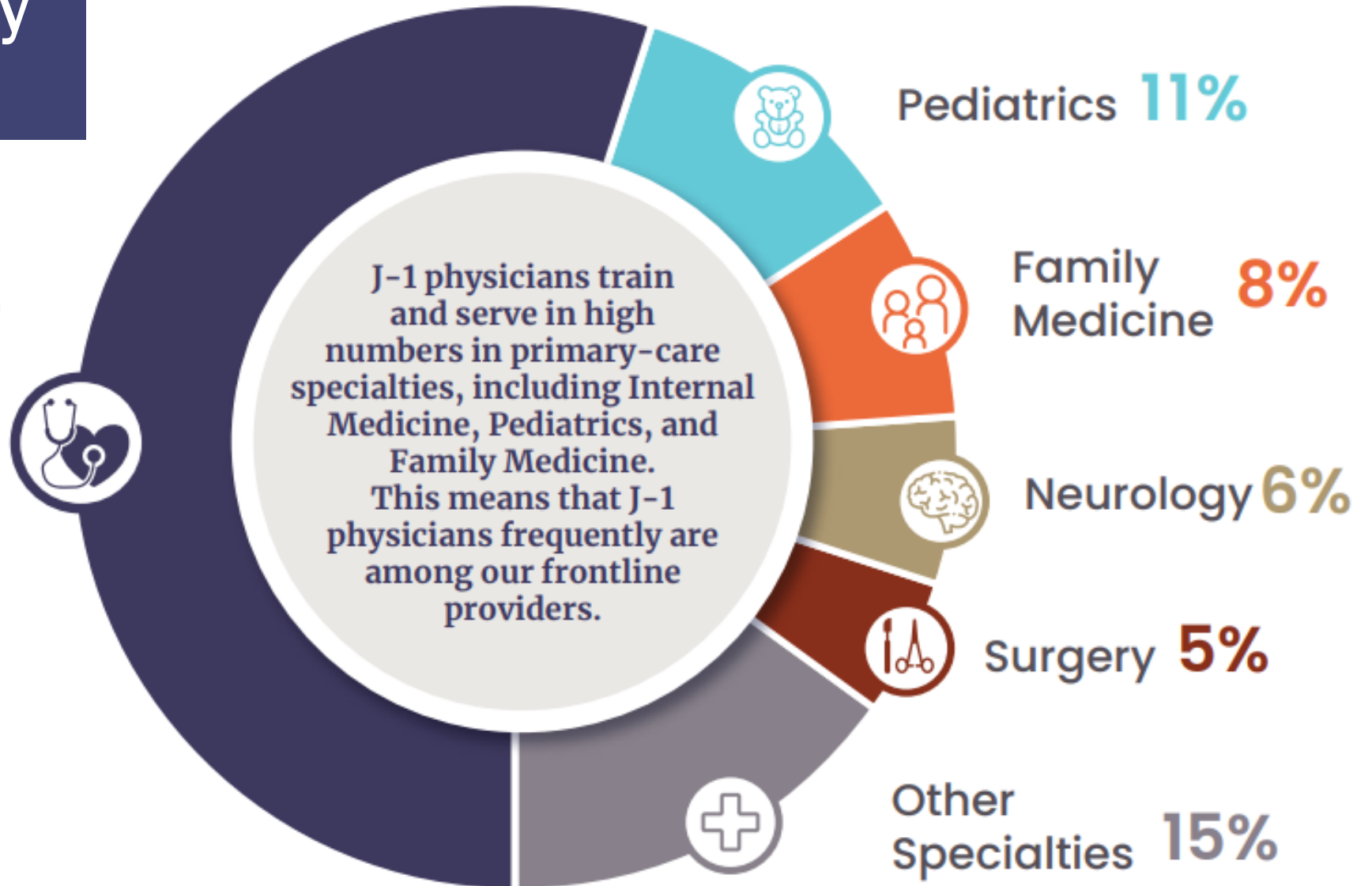
States with Highest Number of J-1s

J-1 physicians train across the nation in 48 states, plus the District of Columbia and Puerto Rico.

New York	2,853	Illinois	754
Michigan	1,227	Ohio	681
Texas	1,195	Massachusetts	640
Pennsylvania	1,034	California	510
Florida	1,029	Connecticut	448
New Jersey	784	Maryland	441

J-1 Physicians by Specialty

Internal
Medicine **55%**



**Specialties listed include related subspecialties.*

J-1 Physicians Serve Vulnerable Americans

By the numbers:

73% of J-1 physicians train in communities ranked High or Medium-High vulnerability by the CDC Social Vulnerability Index, areas facing poverty, disability, lack of insurance, and limited transportation.

That is thousands of physicians serving patients with the fewest alternatives.

Top specialties in which J-1 physicians train:

- Internal Medicine — **55% of J-1 physicians**
- Pediatrics — **11%**
- Family Medicine — **8%**
- Neurology — **6%**
- Surgery — **5%**

The Conrad 30 Program:

This bipartisan program (introduced in Section 2) has placed thousands of physicians in rural hospitals, community health centers, and federally qualified health centers across every state, delivering care where the market alone cannot. The only way that a J-1 physician can remain in the United States is to contribute to a medical underserved area (MUA) of this country through Conrad 30 or otherwise.

Growing state recognition:

To date, 24 states have enacted additional licensure pathways specifically for internationally trained physicians, reflecting growing recognition that IMGs are essential to state-level workforce strategies. Many other states have pending legislation.

Some Pressures Shaping the IMG Pipeline

U.S. Immigration Landscape

U.S. Immigration Landscape

- A dynamic and litigious immigration environment continues to unfold
- Recent and relevant:
 - Evolving “Travel Ban”
 - Elimination of “Duration of Status” for J-1 physicians

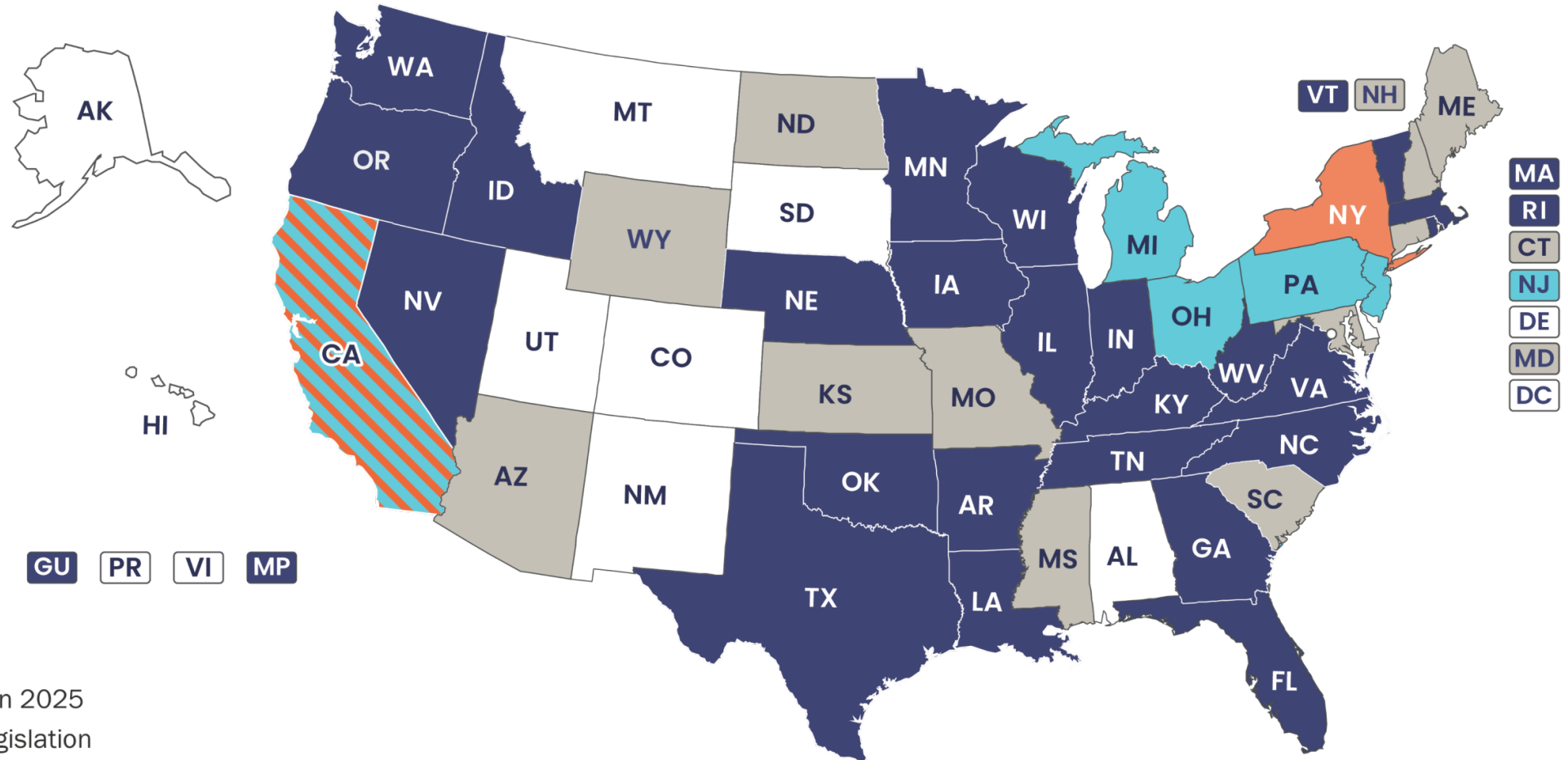
Some Pressures Shaping the Pipeline

Additional Licensing Models for Internationally Trained Physicians (ITPs)

Additional Licensing Models for ITPs, 2023-Current

40+

States with
enacted,
pending, or
proposed legislation



Source: Federation of State Medical Boards
Last updated: June 23, 2026

What are Additional Licensing Models?

Traditional Licensing Pathways – IMGs

- **Common:** ECFMG Certification + 1-3 years accredited US GME + USMLE

OR

- **Uncommon:** “Eminence” pathway for exceptional, highly qualified, and fully trained individuals

Additional Licensing Pathways – ITPs

- Completion of foreign GME “substantially similar” to that recognized in US, *AND*
- Have practiced outside US, *AND*
- Provisional US licensure with supervision, *AND*
- States are retaining ECFMG Certification

Who is an Internationally-Trained Physician (ITP)?

International Medical Graduate (IMG)

- Broader term
- All physicians who received medical education outside US

Internationally-Trained Physician (ITP)

- Small subset of IMGs
- Completed foreign GME “substantially similar” to that recognized in US
- Practiced outside US

Why Additional Licensing Models?

- Lack of access to physicians in many areas
- Anticipated growth in physician shortages
- Insufficient GME training slots to address needs
- States looking for ways to leverage “fully trained” IMGs

“A primary goal of these pathways, reflected in public testimony and written statements submitted by sponsors and supporters in many jurisdictions, is to address U.S. healthcare workforce shortages, especially in rural and underserved areas.”

~Advisory Commission on Additional Licensing Models Guidance Document, August 2025

Advisory Commission on Additional Licensing Models (ACALM)

- Established in response to growing interest among states in additional licensing pathways for ITPs
- Chaired by FSMB, ACGME, and Intealth
- Multiple meetings 2023-2025 and June 2024 national symposium
- Guide and advise state medical boards, state legislators, policymakers, and others
- Does not endorse additional pathways

Purpose: "...to support the alignment of policies, regulations, and statutes, where possible, to add clarity and specificity...to better protect the public...and to advance the safe delivery of quality health care..."

Advisory Commission on Additional Licensing Models (ACALM)

- **Dec 2023** – ACALM established by FSMB, ACGME, and Intealth
- **Jun 2024** – National Stakeholder Forum
- **Feb 2025** – Released Initial Set of Recommendations: Eligibility Criteria/ Entry Considerations for ITPs pursuing a new additional pathway
- **Aug 2025** – Released:
 - Second Set of Recommendations: Assessment/ Supervisory Considerations during provisional licensure period
 - Toolkit for Institutions and Supervisors on Assessment during supervisory period

Moving Forward

- Interest in additional pathways continues to grow
- Discussions to-date have highlighted relevant issues for further consideration, e.g., immigration issues, board certification, credentialing, insurance issues, consensus around key concepts (“substantially similar” training)
- Learning from experience and data

Intealth

A Resource on IMGs

Intealth as a Resource

- Conducting **outreach** to and **education** for lawmakers and policy makers
- Highlighting **unintended consequences**, i.e., when immigration policy becomes health care policy
- Providing **policy guidance and support** for current and prospective J-1 physicians and their U.S. training programs
- Serving as a **resource** on IMGs, foreign national physicians, and related immigration issues for organizations involved in medical education, training, regulation, and research

Thank you!



Information

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